REAL INSURANCE GROUP

MEDICARE MARKETING MACHINE PLAYBOOK

How to get Medicare clients with ads, and keep them for life.

WRITTEN BY

Johnny Brock
and Javier Martinez

HOW TO USE THIS BOOK

The whole machine, written down in order.

Every step, in the order you do it. Read it once from front to back. Then keep it open while you set things up.

PART 1

Who we are, and why we teach this

Who wrote this, and why you can trust it.

PART 2

The Machine

The five parts that turn ads into clients: who you talk to, what you say, your ads, the pages they land on, and your follow-up.

PART 3

Local Market Domination

How to become the go-to Medicare person for dentists, doctors and other offices in your town, without begging for referrals.

PART 4

The Community

Where to get help, ask questions and get started with us.

READ THIS FIRST **Most agents only fix one part, usually the ads. This book fixes all five. Stuck on a word? The plain-English dictionary is on page 4.**

Read it once. Build with it open. Write in the notes pages.

What's inside.

BEFORE YOU START

A–Z **Words you'll see, in plain English** 4
Medicare words, ad words and follow-up words

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WORDS YOU'LL SEE IN THIS BOOK

Medicare words, in plain English.

Insurance is full of confusing words. Here's what they mean. Come back to these pages any time you get stuck.

Original Medicare PARTS A + B
The basic Medicare from the government. Part A is hospital stays. Part B is doctor visits. It pays about 80%. You pay the other 20%, with no limit.

Medigap SUPPLEMENT
A plan that pays the 20% Original Medicare doesn't. Plans have letters: Plan G, Plan N, Plan F. Works with any doctor that takes Medicare.

Network
The list of doctors and hospitals a plan works with. Go outside it and you pay more, or the plan won't pay.

Turning 65 (T65)
People about to turn 65 and get Medicare for the first time.

AEP
The Annual Enrollment Period. Every fall, Oct 15 to Dec 7, people on Medicare can switch plans.

Cross-sell
Selling a second policy to someone who's already your client.

Part D
Drug coverage. A separate plan that helps pay for prescriptions.

Medicare Advantage MA / MAPD
An all-in-one plan from a private insurance company instead of Original Medicare. Often $0 a month and includes dental, vision and drugs. You have to use doctors and hospitals in its network. MAPD means it includes drugs.

SNP / D-SNP
Special Needs Plans. Advantage plans for certain people. Chronic SNP: people with health problems like heart disease or diabetes. D-SNP: people who have both Medicare and Medicaid.

Delayed Part B
People who kept working past 65 and waited to sign up for Part B.

Shield
Our name for extra coverage on top of Medicare: cancer, heart attack and stroke, hospital, home care and dental policies.

FMO
The company agents work under. It gives them insurance companies to sell, training and support. Real Insurance Group is one.

WORDS YOU'LL SEE IN THIS BOOK

Ad words and follow-up words, in plain English.

AD WORDS

Meta
The company that owns Facebook and Instagram. Where you run your ads.

Campaign · Ad set · Ad
The three layers of every Facebook ad, like boxes inside boxes. Big box: goal and money. Middle box: who sees it. Small box: the actual picture or video.

Creative
The picture or video, and the words, in your ad.

CPM
What it costs to show your ad to 1,000 people. Low = strangers. High = people who already know you.

Cost per lead COST PER RESULT
How much you paid for each person who filled out your form.

Landing page
Your own web page that people land on after clicking your ad.

Lead
A person who filled out your form and wants you to contact them.

Angle
A different way of saying the same thing. A picture, a Q&A video and a client story are three angles.

Testing vs growing
Testing means trying new ads to find a winner. Growing (scaling) means spending more on the winners.

Frequency
How many times the same person saw your ad. 1.05 = almost all new people. 1.50 = people who already know you.

Funnel
The path from ad to appointment: ad, web page, form, calendar, text, call. Wide at the top, narrow at the bottom.

Webinar · VSL
A webinar is a free online class. A VSL (video sales letter) is a short video that explains your offer.

FOLLOW-UP WORDS

Speed to lead
How fast you contact a new lead. Text in 1 minute, call in 5.

Nurture
Staying in touch with helpful messages until a lead is ready to buy.

CRM REALCRM / GHL
Software that keeps track of every lead and sends texts and emails for you. Ours is RealCRM, built on GoHighLevel (GHL).

Welcome series
The first few automatic texts and emails a new lead gets.

Show-up rate
Out of everyone who booked a call, how many actually picked up.

CNA
Client Needs Assessment. The questions you ask at the start of a sales call about their health and what they need.



PART ONE

Who We Are

7 years selling Medicare. 4,000+ policies. 1,000+ webinars built.

WHO'S WRITING THIS

Seven years selling Medicare. 4,000+ policies.

My name is **Johnny Brock.** I've sold Medicare for seven years, doing the same daily grind a lot of you are doing. I wrote this for the agent who is struggling, and for the agency owner who wants to grow.



$137K

FIRST 15 MONTHS

I started helping people get on Medicare in 2020. I made $137,000 in my first 15 months.

4,000+

POLICIES SUBMITTED

I've submitted more than 4,000 policies.

#1

TOP SELLER

For four years I sold under Brock Partners, and I was their #1 seller of Medicare Advantage plans.

64

CANCER POLICIES

In one month, I sold 64 cancer policies.

WHAT I DIDN'T HAVE WAS MARKETING SKILLS

I didn't know how to get clients with ads. So I went and learned. I paid $18,000 to learn how to run webinars in 2026 (a webinar is a free online class). That's where I met **Javier Martinez**, who has set up over 1,000 webinars. He was working for the organization I was learning from. We became friends, I told him he should go full time with me, and now we're partners.

Now we work together. Our goal: make Real Insurance Group the best place in the country for Medicare agents to grow. (An FMO is the company agents work under. It gives them carriers, training and support.)

Real experience. Real numbers. No theory.

WHAT I LEARNED ON MY OWN

I'm better at sales. Marketing is my passion.

I am better at sales, but marketing is what I love. So here is how marketing really works. Pay attention. Take notes. Do what we teach and you **will** build a business that keeps growing.





What you get in this book

1. **The exact order to build in**
   First who you talk to, then what you say, then ads, then your pages, then follow-up. Skip one and the next one breaks.

2. **Real numbers from real accounts**
   What we spend, what each lead costs, and what one client is worth. When a number is just an example, we say so.

3. **Scripts you can use tomorrow**
   The questions to ask on a sales call, the exact words to close, and the AI prompts that build your list of local offices.

Sales gets you paid. Marketing gets you scale.



PART TWO

The Machine

Five parts. Every part does its job, every month.

01 Market

02 Message

03 Ads

04 Funnel

05 Backend

WHY MEDICARE IS WORTH MARKETING PROPERLY

The best market in insurance to build a machine around.

Medicare is the best type of insurance to build a client-getting system around. Here are four reasons.

11,000

TURN 65 EVERY DAY

Every single day, 11,000 new people need to pick a Medicare plan. Most can't figure it out alone.

They stay

RETENTION

Medicare clients stick with the agent who helped them. They come back every year.

They need more

LIFETIME VALUE

They also need dental, hospital, cancer, home health and life coverage. One client can buy many policies from you.

They refer

WORD OF MOUTH

Their friends are turning 65 too. A happy client sends you new clients for free.

THAT'S THE WHY That's why. The rest of this book is how.

11,000 a day. Every single day.

MEDICARE IN 60 SECONDS

Every client picks one of two roads.

You need to know this to understand the rest of the book. When someone turns 65, they choose one of these two paths.

ROAD 1

Original Medicare + extras

Part A + Part B
From the government. Hospital and doctor visits. Pays about 80%.

The 20% gap
The person pays the other 20%. No limit. A $100,000 bill = $20,000.

+

Medigap (Plan G, N, F)
Fills the 20% gap. Any doctor that takes Medicare.

+

Part D
Drug coverage. About $10 a month.

No dental, vision or hearing. They need a separate plan for those.

ROAD 2

Medicare Advantage

One all-in-one plan
From a private insurance company like Humana, Aetna or United. Replaces Original Medicare.

Often $0 a month
Usually includes drugs (called MAPD), dental, vision and hearing.

The catch: the network
They have to use the doctors and hospitals that plan works with.

Special versions
SNPs for people with health problems. D-SNPs for people with Medicare and Medicaid.

EITHER ROAD

Both roads leave gaps, like cancer, long hospital stays and home care. That's where the shield comes in (Chapter 5).

THE FRAMEWORK

The Medicare Marketing Machine has five parts.

Think of it like a machine with five parts. If one part is broken, the whole machine stops. Each chapter after this covers one part.

1. **Market** — Who are you trying to reach?
2. **Message** — What do you say so they stop scrolling?
3. **Ads** — How do you set them up and know if they work?
4. **Funnel** — The pages they see after they click.
5. **Backend** — The follow-up that turns a lead into a client.

Clients

Market + Message + Ads + Funnel + Backend = Clients. Every part does its job, every month.

MOST AGENTS

Work on one part.

Usually the ads. Then they wonder why it doesn't work.

THIS BOOK

Builds all five.

In order. The chapters that follow take each part one at a time.

Every part does its job, every month.



CHAPTER ONE

01 Market

Who are you trying to reach?

IN THIS CHAPTER

01 Why your market is the foundation of everything

02 Pick one person, and how to choose your group

03 Three groups almost nobody is calling

04 One campaign for one group

05 Same agent, three groups, three different ads

06 Case study, and the paths from ad to appointment

WHY MARKET COMES FIRST

Your market is the foundation.

Your market is the one group of people you choose to talk to. It sounds simple. But it decides everything that comes after it. Get this wrong, and nothing else works. Get it right, and everything else gets easier.



ONCE YOU PICK YOUR MARKET, YOU KNOW

What to say	What ad to run	What your page says	What to offer	How to follow up
Their worries, in their words	Who's in it, what it shows	A headline made for them	The guide or call they want	What they care about on the call

Pick the person first. Everything else follows.

THE FIRST MISTAKE

"I sell Medicare" is not a market.

Don't try to talk to everyone. Pick one type of person. The more exact you are, the easier it is to write ads that work.

MARKET	WHO THEY ARE
Turning 65	People about to turn 65. The biggest group, so every agent goes after them. Lots of these deals close later, not right away. Still works.
Delayed Part B	People who kept working past 65 and waited to sign up for Part B. Now they're retiring and confused. Great group.
Supplement only	They don't want to hear about Medicare Advantage. They want a Medigap plan, like Plan G or Plan N (or a high deductible plan, or Plan F).
Chronic / SNP	People with heart problems or diabetes. Special plans exist for them (called SNPs). Almost no one advertises to them.
Dental first	Start by helping with dental. It's an easy way in. The Medicare talk comes later.
Hearing first	Same as dental.
Your own book	Your current clients. The cheapest leads you will ever get, because you already paid to get them. If you haven't offered them your other policies yet, do that first.

THE RULE **One campaign. One person. One problem.**



JOHNNY'S TAKE

"If you haven't cross-sold your book, that is the first thing that you should do."

Speak to one person and everyone like them listens.

CHOOSING YOUR MARKET

How to pick your group.

There are many groups you could choose. Ask these five questions about each one. The group that gets the most "yes" answers is where you start.

1. **Can I describe them in one sentence?**
   "Veterans in Mississippi turning 65 this year." If you can't say it in one line, it's too broad.
   CLEAR

2. **Do they have a real problem I can solve?**
   Confused about Part B, worried about the 20% gap, their doctor left their plan.
   PAIN

3. **Is almost nobody else talking to them?**
   Less competition means cheaper leads. Everyone fights over turning 65. Few talk to veterans or people with diabetes.
   ROOM

4. **Do I understand them?**
   Maybe you're a veteran, know sign language, or worked in a dental office. That's an edge nobody can copy.
   EDGE

5. **Are there enough of them where I sell?**
   Check that the group is big enough in your states to keep your calendar full.
   SIZE

NOW WRITE IT DOWN

I help _who (your group)_ **who are worried about** _their main problem_ **so they can** _what they want_**.**

Example: I help veterans turning 65 who are worried about losing their VA benefits so they can keep their care and add the coverage the VA doesn't give them.

WHERE THE ROOM IS

Three groups almost nobody is calling.

Every agent goes after people turning 65. These three groups are just as big, and almost nobody is talking to them.

11.6M

DELAYED PART B



1 in 5 Americans 65+ still working

Almost 1 in 5 Americans over 65 still has a job. Most waited years to sign up for Part B. All of them will need Medicare soon, they're confused, and **nobody is calling them.**

42%

CHRONIC TURNING 65



have a heart condition · 1 in 4 has diabetes

42% of people on Medicare have a heart problem, and 1 in 4 has diabetes. These health problems qualify them for a special plan called a Chronic SNP, and **almost nobody advertises to them.**

28.7M

SUPPLEMENT ONLY



want a supplement, not an Advantage pitch

They don't want Medicare Advantage. They want a Medigap (supplement) plan. **Make ads just for them.**

These are just three. There are dozens more. There are plenty of people to sell to. What most agents are missing is fresh ideas. Fresh ideas get you cheaper leads, cheaper clients and more booked calls.

Sources: Bureau of Labor Statistics 2025 · CMS Medicare Current Beneficiary Survey · CDC · KFF 2025.

The shortage isn't people. It's creativity.

THE MISTAKE WE SEE EVERY WEEK

One campaign for one group.

Most agents try to reach everyone at the same time: people turning 65, veterans, people with diabetes, all in one campaign. It feels like more chances. It's actually the fastest way to waste money.

WHAT MOST AGENTS DO

One campaign, everyone in it

Campaign: "Medicare"

Turning 65 | Veterans | Diabetes | Supplement

The ad says:
"Need help with Medicare?" (generic, so it speaks to nobody)

Result: nobody feels like it's for them. Facebook gets mixed signals. Leads cost more. You can't tell what's working, so you can't grow it.

WHAT WE RECOMMEND

One campaign per group

Campaign 1: Veterans
"Veterans: does your VA care work with Medicare?"

Campaign 2: Turning 65
"Turning 65 this year? 3 decisions to make first."

Campaign 3: Chronic
"Living with diabetes? There's a plan built for you."

Result: each ad speaks to one person. Each one has its own page and its own money. You can see exactly which one works, and grow that one.

THE RULE One campaign for each goal. One goal is one group with one problem. Get one working, then grow it. Then add the next group.

Win one group first. Then add the next.

WATCH THE MARKET CHANGE EVERYTHING

Same agent. Three groups. Three different ads.

Here's why the market comes first. Change the group, and the words, the ad and the page all change with it. Nothing is copy and paste.

	GROUP 1 **Turning 65**	GROUP 2 **Delayed Part B**	GROUP 3 **Veterans**
THEIR WORRY	Too many choices and deadlines. Scared of picking wrong.	Retiring soon. Heard there are late penalties. Confused.	Already have VA care. Don't know if they need Medicare too.
THE HOOK	***"Turning 65 this year? Don't sign anything until you watch this."***	***"Still working past 65? Here's what to do before you retire."***	***"Veterans: here's how Medicare works with your VA care."***
THE AD SHOWS	You at a kitchen table with a checklist.	Someone packing up their office on their last day.	You (or a veteran client) answering the #1 VA question.
THE PAGE HEADLINE	"The 3 Medicare decisions to make before your birthday."	"Retiring soon? Avoid the Part B penalty in 4 steps."	"VA + Medicare: how to use both and fill the gaps."
THE OFFER	Free checklist	Free retirement guide	15-minute VA + Medicare review

THE POINT **You can't write the message until you know the market. That's the next chapter.**

Example ads for teaching only. Always have your ads checked for compliance before they run.

CASE STUDY

He picked one niche, and we had to turn his ads down.

Justin Gray knows sign language. Working with me, he made ads just for deaf people who use sign language (ASL). One small group, one message. It worked so well we had to turn his ads down to **$1 a day.** Each booked appointment cost him about $12. The lesson: the smaller and more exact your group, the better.

~$12

PER BOOKED APPT

Individual results vary. Not a guarantee of future results.

THE PATHS

There's more than one way to run Medicare marketing.

Here are the most common paths from ad to appointment. They all work.

STRATEGY	THE PATH
Facebook lead forms	Ad → they fill a form inside Facebook → you call them
Opt-in forms	Ad → your own web page with a form → you call them
Opt-in to book a call	Ad → form → they pick a time on your calendar
VSL funnels	Ad → a page with a short sales video → they book a call
Evergreen webinars	Ad → sign up → watch a recorded class → book a call
Live webinars	Ad → sign up → join a live class → book a call

Pick one. Master it. Then add the next.

YOUR NOTES

Notes: Market.

MY GROUP
Who is the one group I'm going after?

THEIR PROBLEM
What are they most worried about?

MY EDGE
Why am I the right person for them?



CHAPTER TWO

02 MESSAGE

What do you say so the right person stops scrolling?

IN THIS CHAPTER

01 Why message is what stops the scroll

02 Know what your market is already thinking

03 The four parts of every ad that works

04 The 4 kinds of hooks: curiosity, pain, desire, outcome

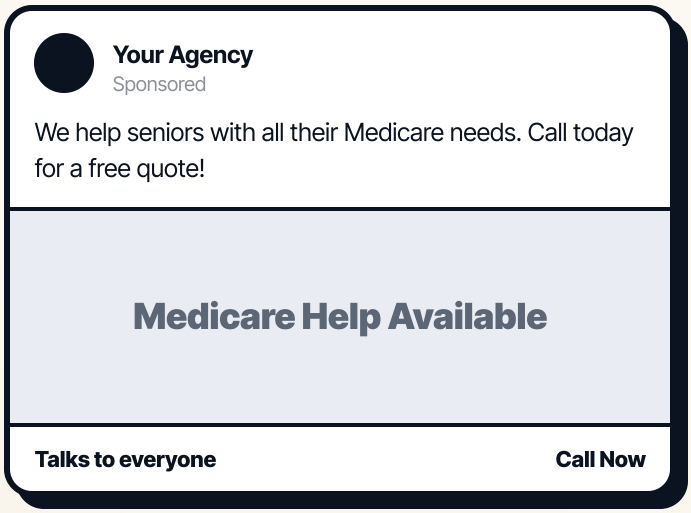
05 Match the hook to the funnel, plus 20 ready hooks

06 Real examples, and write your own ad

WHY MESSAGE MATTERS

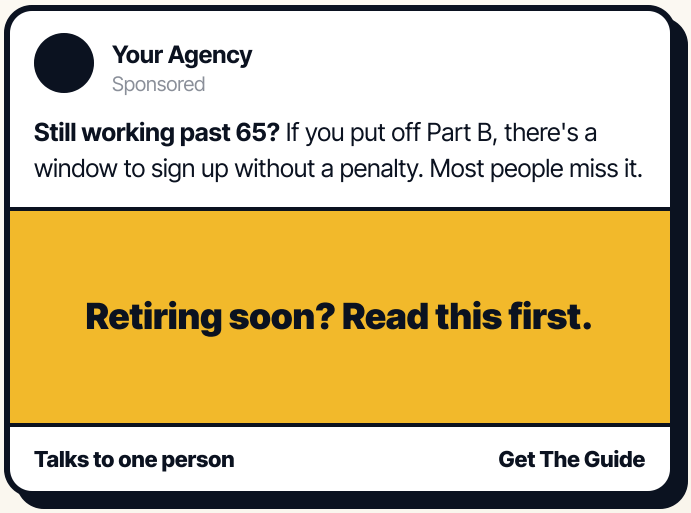
Your message is what stops the scroll.

People scroll past hundreds of posts a day. You get about 2 seconds. In those 2 seconds they decide one thing: "Is this for me?" Your message is how they decide. And you can only answer "yes, this is for you" if you picked a market first.



SCROLLED PAST

No one person feels called out. "Seniors" and "all their needs" could be anyone, so it's no one.



STOPPED AND TAPPED

The right person reads it and thinks, "Wait, that's me." Everyone else scrolls past, and that's fine.

REMEMBER **Market tells you who. Message tells them, "this is for you." You need both.**

Two seconds. "Is this for me?"

BEFORE YOU WRITE A SINGLE WORD

Know what your market is already thinking.

The best ads say what the person is already thinking, in the words they already use. So before you write, fill in these four boxes for your group. Here's an example for people turning 65.

WHAT THEY'RE SCARED OF

- *"What if I pick the wrong plan and I'm stuck with it?"*
- *"I keep hearing about penalties."*
- *"I don't want to lose my doctor."*

WHAT THEY WANT

- *"I just want someone to explain it simply."*
- *"Keep my doctors and my costs low."*
- *"Get this done and stop worrying."*

WHAT THEY'RE ASKING

- *"Do I need Part B if I still have work insurance?"*
- *"What's the difference between Advantage and a supplement?"*
- *"When do I have to sign up?"*

THE WORDS THEY USE

- *"Confused." "Overwhelmed." "All this mail."*
- *"My doctor." "My medicine." "My budget."*
- *Not "MAPD," "SEP" or "formulary."*

WHERE TO FIND THIS

Write down every question your clients ask you this week. Read the comments on Medicare videos and Facebook groups. The exact words people use become your hooks.

Use their words, not insurance words.

THE FORMULA

Every ad that works has the same four parts.

Every ad you write needs all four. If one is missing, the ad won't work as well.

1. **Hook**
   The first line. Name the exact person you want. If they don't think "that's me" right away, they keep scrolling.
2. **Problem**
   Say what they're confused or worried about. Use their words, not insurance words.
3. **Promise**
   Tell them what they'll learn or get for free. Teach, don't sell.
4. **Call to action**
   Tell them the one thing to do next, like "Get The Checklist." Never give two choices.

The four parts in a real ad

PART	WHAT IT SAYS
Hook	Turning 65 this year?
Problem	There are 3 Medicare decisions you have to make before your birthday month, and most people find out about the third one too late.
Promise	Free checklist: The 3 Medicare Decisions Before You Turn 65. It takes 2 minutes to get.
Call to action	**Get The Checklist**

If one line is missing, the ad is missing a job.

THE FIRST LINE OF YOUR AD

The hook is the first line. There are 4 kinds.

The hook is the first thing they read or hear. It has one job: make the right person stop. Every good hook does it in one of four ways. Learn all four, then test them against each other.

Curiosity
Makes them need to know more

Pain
Names the problem they're worried about

Desire
Shows what they want

Outcome
Promises a clear result

SAME PERSON, FOUR HOOKS

CURIOSITY	"The Medicare mistake 1 in 3 people make at 65."
PAIN	"Scared of picking the wrong Medicare plan?"
DESIRE	"Keep your doctor and lower your costs at 65."
OUTCOME	"Pick your Medicare plan in one 15-minute call."

The "1 in 3" is an example to show the style. Only use numbers you can prove.

Same person. Four doors in.



HOOK TYPE · CURIOSITY

Make them need to know.

Open a loop in their head that only your ad can close. They keep watching because they want the answer.

WHY IT WORKS

People hate not knowing. When you hint at a secret, a mistake or a surprise, they stop to find out what it is. Great for reaching new people who've never heard of you.

BEST PART OF THE FUNNEL

TOP | MIDDLE | BOTTOM

Use it to get new people to stop and watch. Then deliver the answer fast, or they feel tricked.

EXAMPLES FOR DIFFERENT GROUPS

TURNING 65	**"Don't sign up for Medicare until you know this one thing."**
DELAYED PART B	**"The Part B rule nobody tells you about when you retire."**
VETERANS	**"What most veterans don't know about Medicare and the VA."**
SUPPLEMENT	**"Why Plan G isn't always the smartest choice."**

FILL IN THE BLANKS

"The surprising thing most your group don't know about topic."

Open a loop. Then close it fast.



HOOK TYPE · PAIN

Name what keeps them up.

Say the exact problem they're worried about. When you name it, they feel understood.

WHY IT WORKS

People move faster to stop a problem than to get something new. When you say their worry out loud, they think, "This person gets it." That builds trust fast.

BEST PART OF THE FUNNEL

TOP | MIDDLE | BOTTOM

Great for people who already feel the problem. Always follow the pain with hope and a clear next step. Never scare people.

EXAMPLES FOR DIFFERENT GROUPS

TURNING 65	"Drowning in Medicare mail and still don't know what to pick?"
DELAYED PART B	"Retiring soon and worried about a Medicare penalty?"
CHRONIC	"Paying too much for your diabetes supplies on Medicare?"
VETERANS	"Not sure if you even need Medicare with VA care?"

FILL IN THE BLANKS

"Feeling or problem about topic? Hope."

Name the worry. Then give them hope.



HOOK TYPE · DESIRE

Show them what they want.

Paint the picture of what life looks like when the problem is solved.

WHY IT WORKS

Some people respond to hope more than fear. A desire hook makes them imagine the good outcome, and they want to know how to get there.

BEST PART OF THE FUNNEL

TOP | MIDDLE | BOTTOM

Works well with client stories. Show a real person who got the result. Only promise what plans can actually do.

EXAMPLES FOR DIFFERENT GROUPS

TURNING 65	"Keep your doctors and pay less for Medicare at 65."
SUPPLEMENT	"See any doctor in the country with no network to worry about."
CHRONIC	"A Medicare plan built around your diabetes care."
DENTAL FIRST	"Get the dental work you've been putting off, on Medicare."

FILL IN THE BLANKS

"What they want without what they're afraid of."

Show them the life they want.

HOOK TYPE · OUTCOME

Promise a clear result.

Tell them exactly what they'll get and how fast. Simple, specific, easy to say yes to.

WHY IT WORKS

People who are close to deciding want a clear next step. An outcome hook tells them exactly what happens if they say yes, so it feels easy and safe.

BEST PART OF THE FUNNEL

TOP | MIDDLE | BOTTOM

Best for booking ads. This is where you ask for the call. Make the result small and specific, like "15 minutes."

EXAMPLES FOR DIFFERENT GROUPS

TURNING 65	"Get your Medicare plan picked in one 15-minute call."
DELAYED PART B	"Your retirement Medicare checklist in 2 minutes."
VETERANS	"A free 15-minute VA and Medicare review."
YOUR OWN BOOK	"Your yearly plan review, done in 10 minutes."

FILL IN THE BLANKS

"Get result in time, with no hassle."

Make the next step small and clear.

PUTTING IT TOGETHER

Match your hook to the stage.

Remember the funnel from the Ads chapter: strangers at the top, people who know you in the middle, people ready to book at the bottom. Each stage needs a different kind of hook. This is how you reach more people and move them down the funnel.

Top STRANGERS	**Use curiosity and pain.** They don't know you yet, so earn their attention. Teach something useful. Don't ask for anything. ***"The Medicare mistake most people make at 65."***
Middle KNOW YOU	**Use pain and desire.** They've seen you. Now build trust: answer their questions, show client stories. ***"Can I keep my doctor on Medicare? Here's the answer."***
Bottom READY	**Use outcome.** They trust you. Now make the next step clear and easy. ***"Book your free 15-minute Medicare review."***

TEST THEM

Run the same ad with 2 to 3 hooks.

Change only the first line. Let Facebook show you which one people stop for.

KEEP WINNERS

The winning hook becomes your next 5 ads.

Found a hook that works? Make more ads with the same idea in new ways.

Right hook. Right stage. Right person.

READY TO USE

20 hooks you can test this week.

Pick your group. Pick a hook. Make it sound like you. Test two or three at a time.

TURNING 65

1. "Turning 65 this year? Don't sign anything until you watch this."
2. "3 Medicare decisions you have to make before your birthday."
3. "Advantage or supplement? Here's how to choose in 2 minutes."
4. "The Medicare deadline most people find out about too late."
5. "Your Medicare checklist for turning 65."

DELAYED PART B

1. "Still working past 65? Read this before you retire."
2. "Leaving your job? Here's what happens to your health insurance."
3. "The Part B window you can't afford to miss."
4. "Retiring this year? Do these 4 things first."
5. "Work insurance ending? Here's your Medicare plan."

VETERANS

1. "Veterans: do you need Medicare if you have VA care?"
2. "How Medicare and the VA work together."
3. "What the VA doesn't cover, and how to fill the gap."
4. "A free 15-minute Medicare review for veterans."
5. "Veterans turning 65: watch this first."

CHRONIC AND SUPPLEMENT

1. "Living with diabetes? There's a Medicare plan built for you."
2. "Heart condition? Your Medicare plan should cover this."
3. "Want any doctor, no network? Watch this."
4. "Plan G vs Plan N: which one saves you more?"
5. "Is your supplement going up again? Here's what to do."

Always have your ads checked for compliance before they run. Never say or suggest you are Medicare or the government.

Steal the idea. Make it sound like you.

WHAT IT LOOKS LIKE IN THE FEED

Same four parts. Two real formats.

Here are two simple Facebook ads that use one picture and a few lines of text (we call these static image ads). Start with these. They're the easiest to make and they work. Move to video later.

JB **Your Agency**
Sponsored

Turning 65 this year? There are 3 Medicare decisions you have to make before your birthday month. Most people find out about the third one too late.



FREE CHECKLIST
The 3 Medicare Decisions

Get The Checklist

JB **Your Agency**
Sponsored

Dental on Medicare? Most plans cover less than people think. Here's how to check yours before your next cleaning.



FREE GUIDE
Medicare Dental, Explained

Learn More

Examples only. These belong to Real Insurance Group, so don't copy them. Make your own using your own ideas.

IDEA FOLDER

Picture ad ideas

A big folder of picture ad ideas you can learn from.

drive.google.com/drive/folders/1wBvW_gQjr8HgI7T_D7-FWvt3bXmOh7JG

IDEA FOLDER

Video ad ideas

A big list of video ad ideas and scripts.

docs.google.com/document/d/15nZXZ5UNB7I4E7N1Bf-0lHHimEUN1Rkknhgw4RG00eY

YOUR TURN

Write your ad in 10 minutes.

Fill this in for the one group you picked in Chapter 1. When you're done, you have a complete ad. Then write two more hooks and test all three.

Your group From Chapter 1	
What they're scared of In their words	
Hook type Curiosity, pain, desire or outcome	
1 · Hook The first line	
2 · Problem What they're worried about	
3 · Promise What they get for free	
4 · Call to action One button	
2 more hooks to test Different types	

CHECK IT Read it out loud. Would someone in your group think "that's me" in the first line? If not, rewrite the hook.

One group. One worry. One button.

YOUR NOTES

Notes: Message.

THEIR WORDS
What do they say when they're worried?

MY HOOKS
Which 3 hooks will I test first?

MY OFFER
What free thing will I give them?



CHAPTER THREE

03

Ads

How do you set them up, and how do you know if they work?

IN THIS CHAPTER

01 How Facebook ads work, in plain English

02 Every ad has a job: find people, build trust, or book calls

03 How to set up your ads, step by step

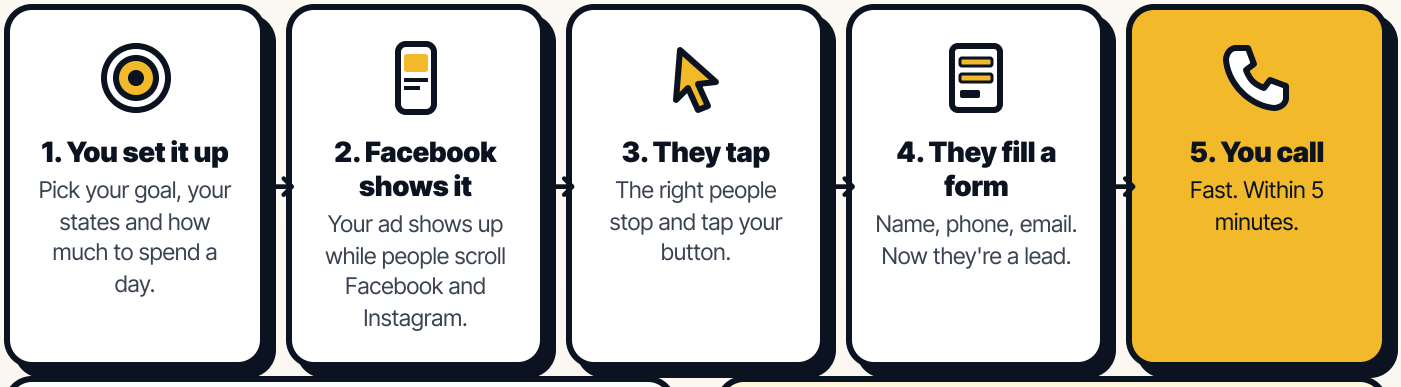
04 Test new ads in one place, grow the winners in another

05 The four numbers to check, and a real account at $1.64 a lead

FACEBOOK ADS IN 60 SECONDS

How a Facebook ad turns into a phone call.

If you've never run an ad before, start here. This is the whole thing, in five steps.



HOW YOU PAY

You pay Facebook to show your ad.

Not per sale. You set a daily amount, like $20 a day. Facebook spends it showing your ad to the people most likely to respond.

WHO SEES IT

Your ad picks your people.

You don't pick interests. If your ad says "Turning 65 this year?", Facebook learns to show it to people turning 65.

WHAT THIS CHAPTER TEACHES

What kind of ads to make, how to set them up, how to find the ones that work, how to spend more on the winners, and which four numbers tell you if it's working.

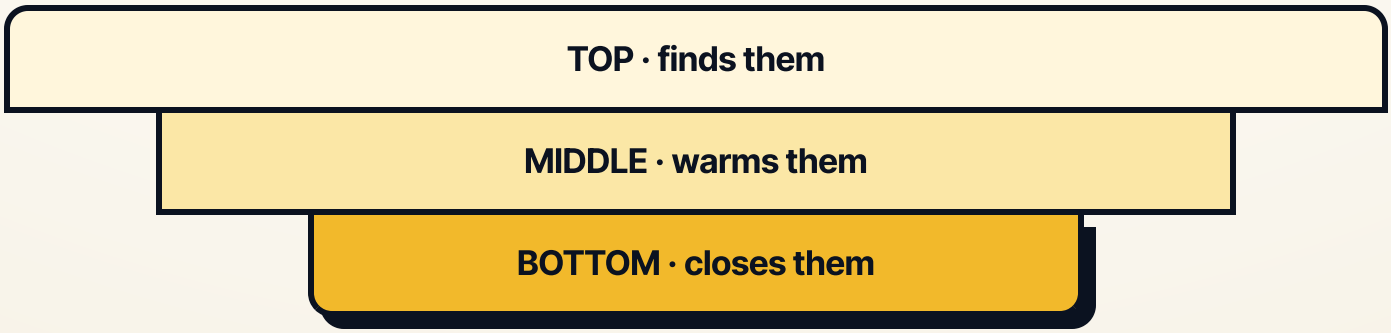
Set it up. Let it find people. Call them fast.

CHANGE THE QUESTION

Stop asking "is this ad working?" Ask "what job is it doing?"

Think of a funnel: wide at the top, narrow at the bottom. Lots of strangers go in the top. A few ready buyers come out the bottom. Every ad has a job at one level of that funnel. Judge each ad by its job.

STAGE	WHO THEY ARE	THE AUDIENCE	THE ADS
Top	Never seen you. Don't know you.	Lots of people. Cheap to reach.	**Finding ads**
Middle	Seen you a few times.	Watched your videos or engaged.	**Trust ads**
Bottom	Seen you 7 to 17 times.	Already know you. Seen your ads many times.	**Booking ads**



The top finds them. The middle warms them. The bottom closes them.

WHAT EACH STAGE LOOKS LIKE

One job per stage. Here's what to make.

TOP · STOP THE SCROLL

PROSPECTING

Who: people who fit your ideal client (right age, right area) but have never heard of you. **The video:** one quick, useful tip, under 60 seconds. The hook is the first line, about their life, not your agency.

> **"Turning 65? Don't sign anything until you watch this."**

Its one job: get noticed and earn the next watch. Don't ask for the call yet. Teach first, sell later.

MIDDLE · BUILD TRUST

TRUST

Who: people who already watched your videos, liked, or commented. **The video:** you on camera answering a real question, or a real client telling their story.

> **"Can I change my plan if my doctor leaves? I get asked this every week. Here's the answer."**

Its one job: trust. By the end, they feel like they know you. The question your clients ask every week is your next ad.

BOTTOM · GET THE BOOKING

CLOSING

Who: people who have seen you many times, visited your page, or started your form. **The video** is short and direct: who you help, what happens on the call, and a clear button to book.

> **"Your 15-minute Medicare review. Pick a time that works for you. No pressure."**

Its one job: the booking. Now is when you ask. Not before.



JOHNNY'S TAKE

"The question your clients ask you every week is your next ad."

Teach first. Sell later.

STARTING OUT?

Start at the bottom, then work up.

Your first clients come from the people who already want to talk. Build in this order.

1

The people ready now. Closing ads.

2

Then build trust. Q&A and client stories.

3

Then fill the top. Quick tips that bring in new people.

Booking ads catch people who are ready to talk today. Trust ads help people who need to see you a few more times. Finding ads bring in new people, so you never run out.

Every Facebook ad has three layers.

Think of boxes inside boxes. (Meta is the company that owns Facebook and Instagram.)

LEVEL	WHAT IT CONTROLS
Campaign	**The big box.** What you want (like leads) and how much money to spend.
Ad set	**The middle box.** Who sees it, and where they fill out their info.
Ad	**The small box.** The actual picture or video and words people see. This decides who stops and pays attention.

Close first. Then build trust. Then fill the top.

THE AD SET

Keep the ad set simple.

1 · CONVERSION

Choose your event

Tell Facebook what you want people to do: fill a form, visit your site, call, or message you. Pick "leads." Then Facebook goes and finds people who fill out forms.

2 · BUDGET

Not here

Set your budget on the big box (the campaign), not here. Facebook moves the money to what works.

3 · AUDIENCE

Go broad

Pick the states you sell in. Don't pick interests or other filters. Let Facebook find the people.

INSTANT FORM OR LANDING PAGE?

Here's the trade.

INSTANT FORM · INSIDE FACEBOOK

Upside: more leads, cheaper.

Downside: easy to fill out without thinking.

The fix: choose the "Higher intent" setting. It makes people double check their info before they send it. Fewer fake leads.

LANDING PAGE · YOUR OWN PAGE

Upside: fewer leads, better leads.

Downside: costs more per lead.

The fix: match the ad. Same words, same promise. If the page looks different, they leave.

A cheap lead who never answers the phone isn't cheap.

THE AD

This is your targeting.

You didn't pick interests, so the ad itself picks your people. If your ad says "Turning 65?", Facebook learns to show it to people turning 65.

FORMAT

Single video or single image.

PRIMARY TEXT

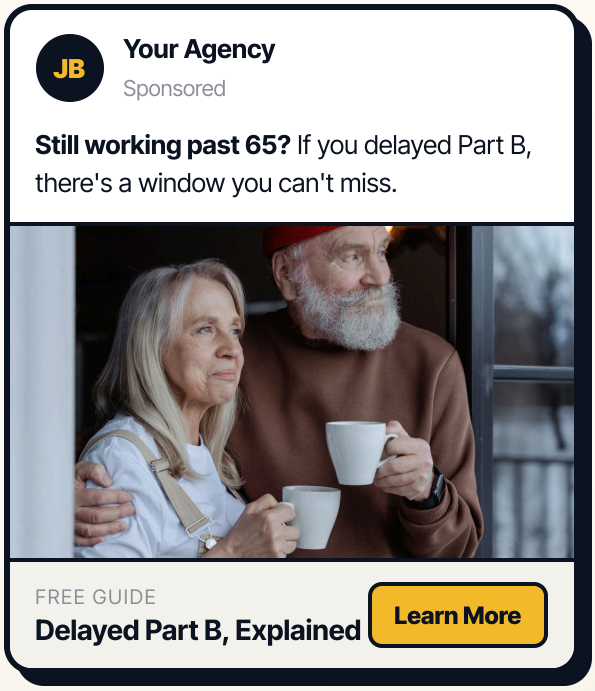
Call out the person and the problem.

HEADLINE

The promise.

CALL TO ACTION

The button. Use "Learn More" or "See Details." You can also use "Book Call" and connect your calendar.



WHILE YOU TEST **Put only one or two ads in each middle box. Give each middle box one idea. Test AI ads, Q&A videos and client stories in separate boxes.**

Who the ad talks to is who Meta finds.

THE SYSTEM

Test hard. Scale the winners.

Run two separate campaigns (big boxes). One is for trying new ads. One is for spending more on ads that already work. Never mix them.

CAMPAIGN 1

Testing campaign

Lots of new ads. It has its own money, so new ads always get a fair shot. **Its job is to find the winners.**

CAMPAIGN 2

Scaling campaign

Only ads that already work, and most of your money. **Its job is to spend more on the winners.**

WHY YOU NEVER MIX THEM

Put new ads in with your winner, and your winner eats the whole budget.



The winner gets most of it, new ad A gets a little, new ad B almost none, and new ad C is never shown. Was new ad C your next winner? You'll never know. When new ads have their own campaign and their own money, every one gets a real chance. Then you decide.

Only winners move over.

INSIDE THE TESTING CAMPAIGN

Test one angle per ad set.

An angle is a different way to say the same thing to the same person. Example: one ad is a picture, one is you answering a question, one is a client's story. Give each angle its own ad set. When one wins, you know exactly why.

START HERE

AD SET 1

Static image

One strong line, one picture. Easiest and most effective. Then graduate to video.

AD SET 2

Q&A

You on camera answering a real question people ask you.

AD SET 3

Testimonials

Real clients telling their story in their own words.

AD SET 4

AI ads

The same script, but the video is made with AI tools.

Keep each ad set tight.

SIX ADS IN ONE AD SET

Eats it all · · · · · ·

Put six ads in one ad set and the budget is spread too thin: one ad eats it all and five never get a real test.

ONE OR TWO ADS

Ad 1 Ad 2

With one or two ads, the budget stays focused and both get enough money to prove themselves.

Fewer ads per ad set gets you a real answer, faster.

HOW MUCH SHOULD YOU TEST WITH?

Test with what one client is worth.

Say one new client pays you $600 (this is just an example). Then let a new ad spend up to $600 before you decide it doesn't work.

ONE CLIENT IS WORTH		YOUR TEST BUDGET
$600	=	**$600**
in commission (example)		before you judge it

BE HONEST WITH YOURSELF

Not willing to spend what one client is worth to find out? Then you're not ready for ads yet. If you get one client, you made your money back, and now you know the ad works.

ONCE YOU HAVE A WINNER

Split your money 70/30.

70%	30%
Growing: ads that already work. This money pays the bills.	**Testing.** New ads and angles.

30% testing: new ads and new ideas. Every winning ad gets old and stops working. The 30% is how you find the next winner before that happens.

Break even on one client, and you bought the answer.

HOW TO SCALE A WINNER

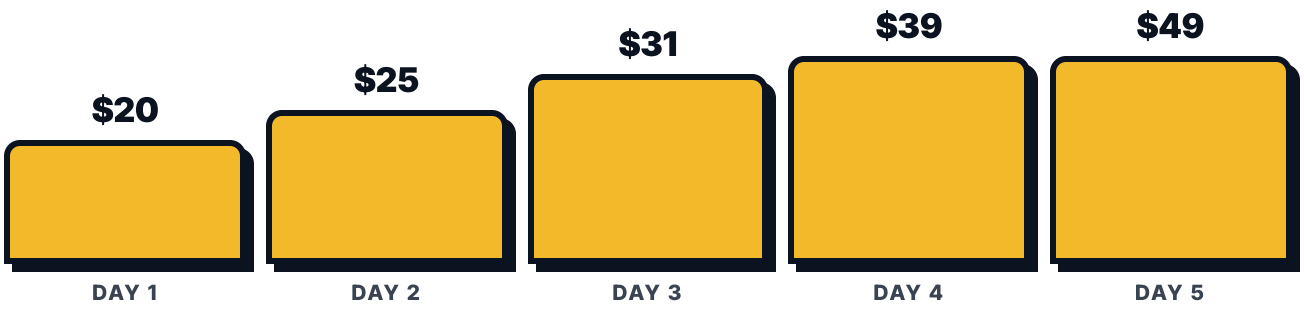
Move it right. Raise it slow.

MOVE THE WINNER, KEEP THE PROOF

Move the winner, and keep its likes and comments.

When an ad wins, move it to your growing campaign with the "Use existing post" button. It keeps all its likes and comments. If you rebuild it from scratch, it starts at zero likes, and people trust it less.

Add about 25% more money a day.



WHY NOT JUST DOUBLE IT?

Twice the money does not mean twice the leads. Big jumps confuse Facebook and the ad can stop working. Small steps keep it working.

FILLS ONE ANGLE AT A TIME

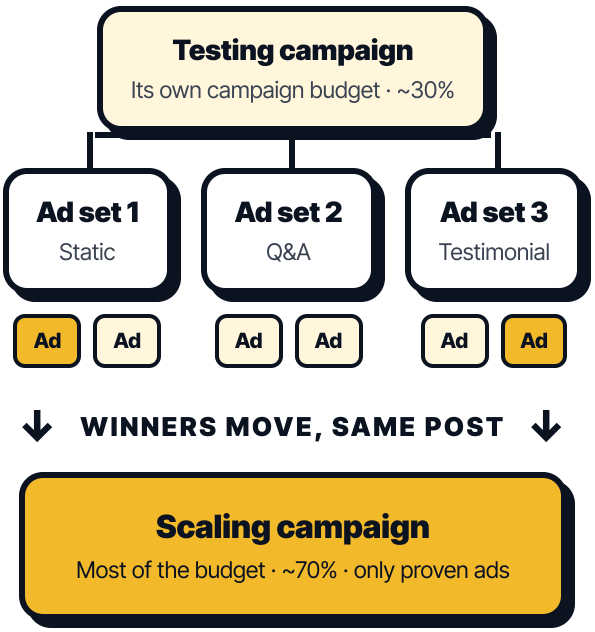
Your growing campaign fills up one idea at a time. If your Q&A video and your picture ad win, they move over. You can only grow as fast as you find new winners.

Go slow. Killing a winner costs more than waiting.

PUT IT TOGETHER

A winning campaign structure.

Here's the whole setup on one page. New ads get tested at the top. Only the winners move down, keeping their likes and comments.



1
testing campaign, with its own budget

3
ad sets, one angle each

1–2
ads per ad set

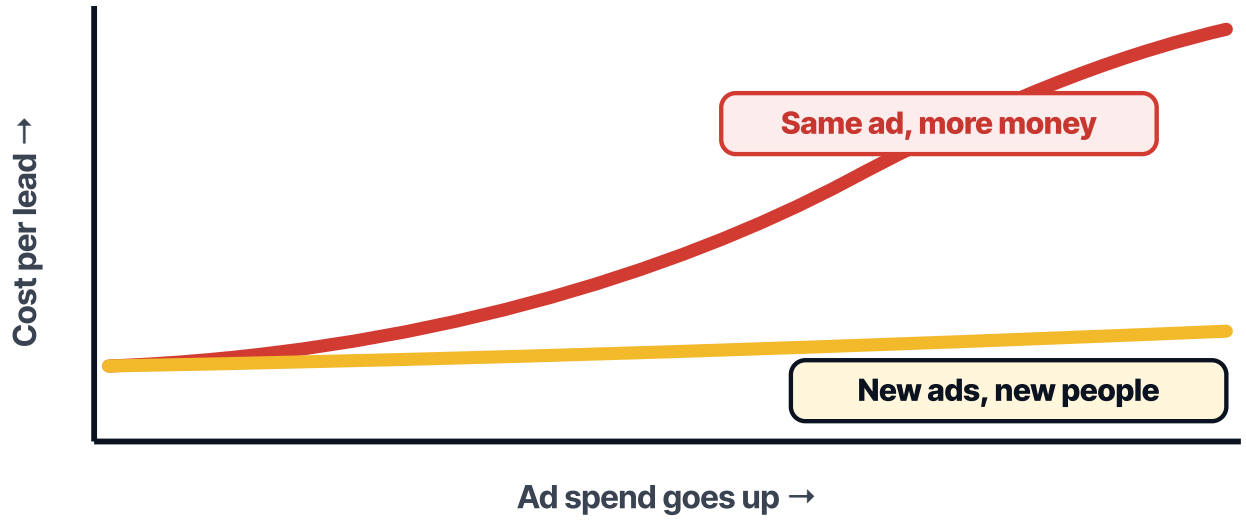
→
Winners move to the scaling campaign, which holds most of the budget

Test in one campaign. Scale in another.

WHY YOUR ADS STOP SCALING

Same ad, more money, higher cost.

You find an ad that works. You spend more on it. Then each lead starts costing more. That's not bad luck. It happens to everyone.



ILLUSTRATION, NOT REAL DATA

WHAT HAPPENS

One ad runs out of people.

Facebook already showed it to the people most likely to say yes. To spend more, it has to show it to people who care less. So each lead costs more.

THE FIX

New ads find new people.

Every new idea reaches a new group of people. Then your other ads walk them from stranger, to fan, to booked call.

You don't scale an ad. You scale your creative.

HOW MANY ADS DO YOU NEED?

More than you think. Every spend level needs more creative.

More money means Facebook needs more people to show your ads to. Reaching more people takes more ideas. So the more you spend, the more new ads you need.

MONTHLY SPEND	CREATIVE OUTPUT	WHAT IT LOOKS LIKE
$1,500	**~20 ads** in rotation	One winner carrying the results. The rest are new ads testing to beat it.
$10,000	**10 to 15 new ads** every month	More winners running at once, with a steady stream of fresh ads launching behind them.

AT $1,500 / MO

1 winner, the rest testing

Winner | New | New | New | New | New | New | New

AT $10,000 / MO

More winners, more new ads

Winner | Winner | New | New | New | New | New | New | New | New

THE RULE Turn off losing ads fast. Keep adding new ones. You can't find a winner if you don't try enough ads.

Every new ad is trying to beat your best one.

WHERE THE NEW PEOPLE GO

New ads bring new people. The funnel turns them into leads.

Reaching new people only works if you have a clear path that takes a stranger all the way to a booked call. New ads bring people in. The rest of the funnel does the work.

Prospecting TOP · COLD	**Never seen you.** Lots of people, cheap to reach. The only job is to get noticed. Don't ask for anything yet.
Middle WARM	**Seen you, engaged.** Build trust. Q&A, client stories, the questions they're already asking.
Bottom HOT · CLOSING	**Seen you 7 to 17 times.** Now ask. Clear offer, clear button, book the call.

YOUR WEEKLY CREATIVE RHYTHM

- [ ] **Check the four numbers** on the next page.
- [ ] **Cut losers fast.** Don't wait for a bad ad to turn around.
- [ ] **Put new ads in your testing campaign.** It has its own money, so they always get a fair shot.
- [ ] **Move winners to your growing campaign.** Use the same post. About 70% of your money there, 30% on testing.
- [ ] **Refill.** Every ad you cut gets replaced with a new angle.

READING YOUR ADS

The four metrics that matter most.

When you open Facebook Ads Manager (the page that shows your ad results), there are dozens of numbers. Only look at these four. Ignore the rest for now.

1 · SPEND

Where the money went

How much money each ad used. Facebook gives the most money to the ad it thinks will work best.

2 · CPM

Who you reach

How much it costs to show your ad to 1,000 people. Cheap means strangers. Expensive means people who already know you.

3 · FREQUENCY

Where it sits

How many times the same person saw your ad. It tells you if you're reaching new people or the same people again.

4 · COST PER RESULT

Is it doing its job?

What you pay for each lead, or even better, for each booked call. This tells you if the ad is working.

SPEND · META IS TALKING



If one ad gets 70% of the money and another gets 3%, Facebook is telling you something. The top ad is usually finding new people, so don't turn it off just because its leads cost more.

LOW CPM (CHEAP)

Cheap to reach means **strangers who don't know you yet.** That's normal for finding ads.

HIGH CPM (EXPENSIVE)

Expensive to reach means **people who already know you.** That's normal for booking ads. It's not a problem.

FREQUENCY + COST PER RESULT

Frequency: how many times the same person saw your ad.

1.05	1.15	1.25	1.50
Brand-new people	Mostly prospecting	The gray zone	People who know you

Check it for one day, not the whole month. Higher number = people who already know you.

Cost per result: judge it against its job.

Don't compare a finding ad to a booking ad. A finding ad at $7 a lead can be doing a better job than a booking ad at $3.

FINDING AD

$7

Finding new people. Doing its job.

BOOKING AD

$3

Cheaper, but only because the finding ad warmed them up first.

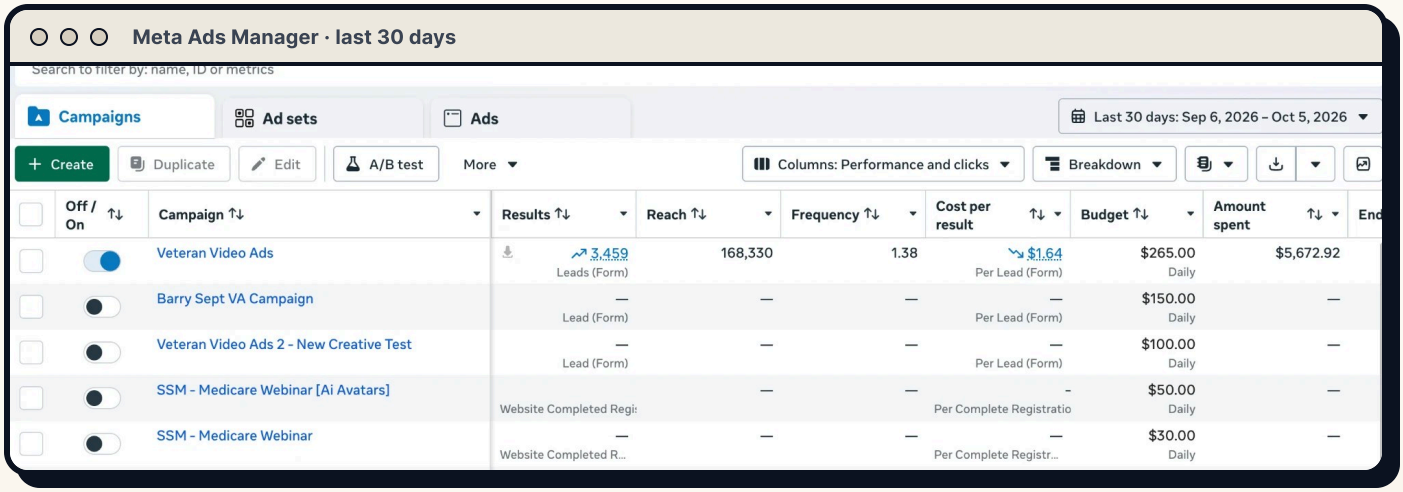
THE REAL NUMBER

The number that really matters: how much you pay for each booked call.

PROOF

Real account: 3,459 leads at $1.64 each.

A real client of ours, running ads to veterans, over 30 days (Sept 6 to Oct 5, 2026).



3,459	$1.64	$5,672	168,330	1.34
LEADS	PER LEAD	SPENT	REACHED	CPM

READ IT LIKE A PRO

Frequency was **1.38**. That means most people only saw it once or twice. It's reaching new people, which is exactly its job.

Individual results vary. Not a guarantee of future results.

Judge the ad by its job.

YOUR NOTES

Notes: Ads.

MY BUDGET
How much a day? What is one client worth?

MY ANGLES
Which 2 to 3 angles will I test?

MY NUMBERS
Spend, CPM, frequency, cost per lead this week:



CHAPTER FOUR

04

Funnel

The pages people see after they click your ad.

IN THIS CHAPTER

01 The 7 steps from ad to appointment

02 What people think and feel on each page

03 How to find where people drop off

HOW A FUNNEL CONVERTS

Every step is intentional.

A funnel is the path from your ad to a booked appointment. It's a few simple pages and messages. Each step has one job: get the person to take the next step.

STEP	WHAT IT DOES IN THEIR HEAD	EXAMPLE
1 · Ad	**Curiosity.** Stop the scroll.	"Get The Checklist"
2 · Landing page (your web page)	**"This is for me."** The page says the same thing as the ad.	Same words as the ad
3 · Form	**A small yes.** A short, easy form.	Name, phone, email, turning 65 when?
4 · Thank-you page (after the form)	**Book now.** Ask while they're still excited.	"Book Your Time"
5 · Calendar	**Their choice.** They pick the time.	Tue · 10:00 AM
6 · Text + email	**Trust.** They know who's calling.	"Hi Mary, it's Johnny's office. You're booked for Tue at 10."
7 · Appointment	**They show up.** Because they asked for it.	Medicare review, confirmed



Every step does one job in their head.

WHY IT WORKS

What people think on each page.

LANDING PAGE

Message match

Your page must say what the ad said. If it doesn't match, people feel tricked and leave.

FORM

Small yes first

Keep the form short. Every extra question is one more reason to quit.

THANK-YOU PAGE

Strike while it's warm

Right after they fill the form, they're the most interested they will ever be. Ask them to book a call right then.

CALENDAR

Let them choose

When they pick the time, they feel it's their appointment. Only show the next few days.

CONFIRMATION

No strangers

Text them right away from your phone number, so your call isn't a surprise.

THE WHOLE FUNNEL

One next step

Each page asks for just one thing. Too many choices and people pick nothing.

WHAT THEY'RE THINKING AT EACH STEP

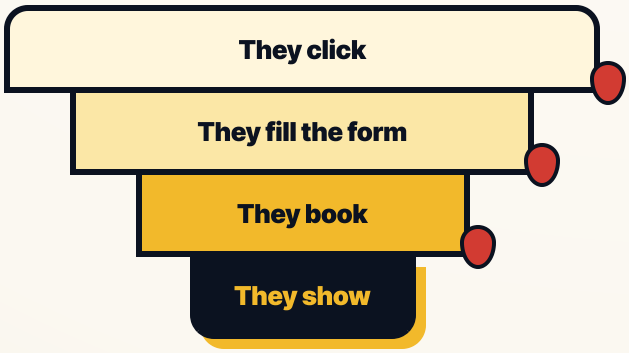
AD	PAGE	FORM	THANK YOU	CALENDAR	TEXT
"Wait, that's me."	*"This is for me."*	*"That was easy."*	*"Let's book it now."*	*"Tuesday works."*	*"Oh, it's Johnny's office."*

One page. One job. One next step.

SOMETIMES IT'S NOT THE AD

Find the leak.

Before you turn off an ad, check where people stop. The problem might not be the ad at all.



THEY CLICK, BUT DON'T FILL OUT THE FORM

The leak is the page.

The page doesn't match the ad, or asks too much.

THEY FILL THE FORM, BUT DON'T BOOK

The leak is the booking.

You called too late, or gave them no reason to book now.

THEY BOOK, BUT DON'T SHOW

The leak is the reminders.

No reminder texts, and no follow-up when they miss it.

REMEMBER The ad can only bring them to the door.



JOHNNY'S TAKE

"Sometimes it's not the ad. The ad can only bring them to the door."

Fix the leak before you kill the ad.

YOUR NOTES

Notes: Funnel.

MY PATH
Ad to form to calendar to text. What's missing?

MY PAGE
Does my page say what my ad says?

THE LEAK
Where are people dropping off?



CHAPTER FIVE

05

Backend

Your ad doesn't make you money. Your follow-up does.

IN THIS CHAPTER

01 Why follow-up matters as much as ads

02 Call fast, then keep in touch with texts and emails

03 Get more people to actually show up to their call

04 The software that does it for you, and what one client is worth

05 Shield Packaging: how to sell extra coverage without being pushy

WHERE THE MONEY IS MADE

The back end is just as important as the front end.

THE FRONT END

Gets you the lead

Everything up to the form: who you target, your message, your ads, your pages. Most agents spend all their time here.

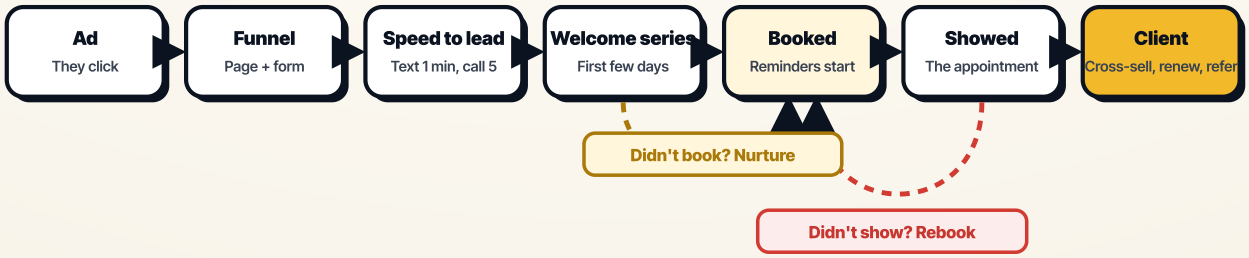
THE BACK END

Turns the lead into money

Everything after the form: calling fast, staying in touch, reminders, rebooking no-shows, selling more policies, and getting referrals. This is where the money is made.

THE CUSTOMER JOURNEY

What it actually looks like.



TWO LOOPS SEND PEOPLE BACK TO "BOOKED"

LOOP 1 · DIDN'T BOOK YET?

Long-term nurture

Every week, for months, until they're ready. Back to booked.

LOOP 2 · DIDN'T SHOW?

No-show recovery

A text within minutes. Rebook them. Back to booked.

Every step has to happen automatically, or it won't happen at all. Too many agents can say: "My ad worked. Everything after it didn't."

The ad gets the lead. The back end gets paid.

SAME AD. SAME 100 LEADS.

Four times the clients.

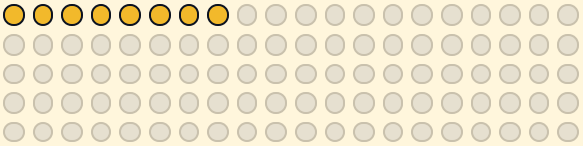
	AGENT A · NO BACK END	AGENT B · WITH A BACK END
Speed to lead	Calls the next day	Text in 1 minute, call in 5
Reminders	None	Night before and morning of
No-shows	Gone	Rebook text in minutes
Not ready yet	Forgotten	Texts and emails for 90 days

AGENT A · 100 LEADS, NO BACK END



2 clients **$1,388**

AGENT B · SAME 100 LEADS, WITH A BACK END



8 clients **$5,552**

Example numbers.

The ad didn't change. The back end did.

SPEED TO LEAD

The first five minutes decide the lead.

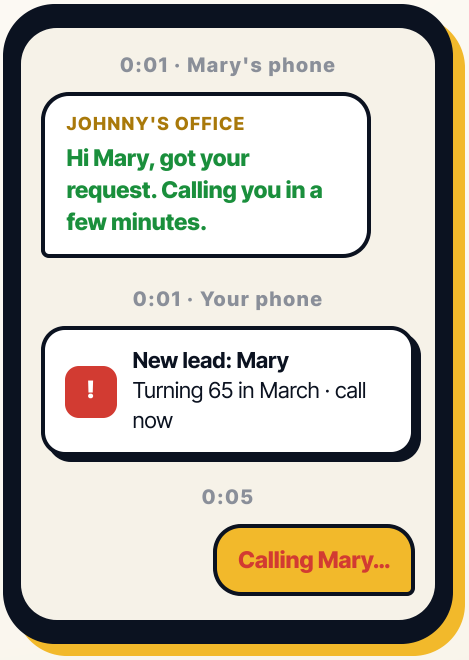
A text goes out on its own in the first minute. Then you make the call.

0:00 **Mary fills out your form.**

0:01 **She gets a text:** "Hi Mary, got your request. Calling you in a few minutes."

0:01 **Your phone buzzes** with a new-lead alert.

0:05 **You call.** She still remembers filling it out.



CALL IN 5 MINUTES

You aren't a stranger.

On the booked call, she already knows who you are.

WAIT UNTIL TOMORROW

You're a stranger.

She's forgotten the form, and maybe talked to someone else.



JOHNNY'S TAKE

"Now on the booked call, you aren't a stranger. Wait until tomorrow and you are a stranger."

Five minutes, not tomorrow.

FOLLOW-UP

Two sequences, two different jobs.

THE WELCOME SERIES · FIRST FEW DAYS

Automatic texts and emails right after they sign up.

Turn a lead into a booked call

DAY 0
Here's your checklist. Here's who I am.

DAY 1
The 3 Medicare mistakes that cost people the most.

DAY 2
A real story from a client like you.

DAY 3
"Want me to walk you through it? Pick a time."

LONG-TERM NURTURE · EVERY WEEK, FOR MONTHS

"Nurture" means staying in touch until they're ready.

Stay close until they're ready

- [ ] **Weekly:** one useful tip or answered question.
- [ ] **Timing:** reminders before their 65th birthday and before AEP (the fall window when people can change plans, Oct 15 to Dec 7).
- [ ] **Proof:** client stories, so they trust you before they call.
- [ ] **Check-in:** "Still looking? Want me to call?"

EVERY WEEK, FOR MONTHS



Gold = a useful tip or a client story. Black = reminders before their birthday and before AEP.

TIP This should go on for 90 days. Some book in a day. Some take six months. Keep both.



JOHNNY'S TAKE

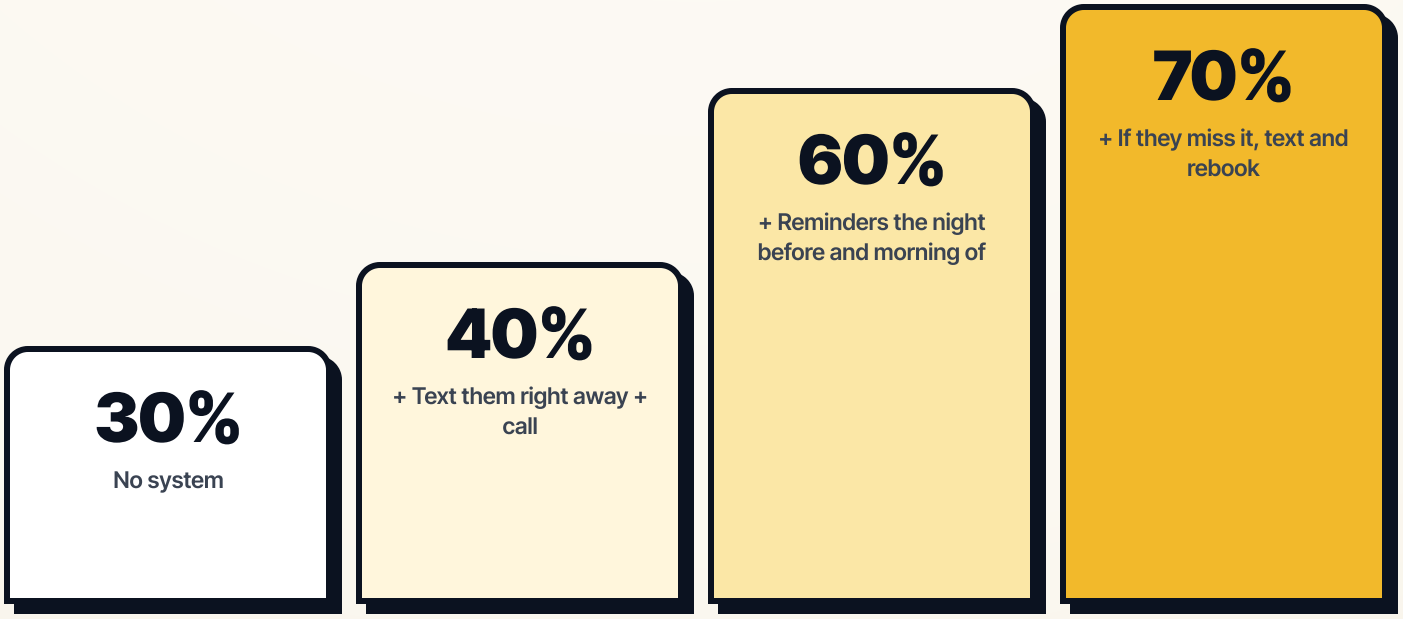
"Some book in a day. Some take six months. Keep both."

Some book in a day. Some take six months.

SAME ADS, MORE CONVERSATIONS

Get more people to show up.

Your "show-up rate" is how many people who book a call actually answer. Each step below gets more of them on the phone.



DO THE MATH

Out of 100 booked calls, that's 40 people or 70 people. Same ads, same leads, same budget: **30 more conversations every 100 bookings,** with no new ads.

30 more conversations. Zero new ads.

A FUNNEL ISN'T BUILT ONCE

It's optimized every week.

EVERY WEEK · 15 MINUTES

Fix the weakest step

Check these three numbers. Fix the worst one. Only that one.

- [ ] **Form rate:** how many visitors fill out the form?
- [ ] **Booking rate:** how many leads book a call?
- [ ] **Show-up rate:** how many show up?

EVERY MONTH · ONE TEST

Make one thing better

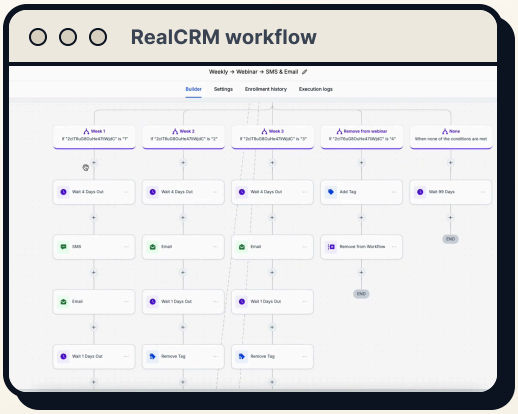
Pick one and test it.

- [ ] **The page:** a new headline.
- [ ] **The follow-up:** a new reminder or message.
- [ ] **The ads:** which ads made clients, not just leads?

YOU CAN'T DO THIS BY HAND

This is where RealCRM comes in.

A CRM is software that keeps track of every lead and sends texts and emails for you. Everything in this chapter runs inside one. Ours is RealCRM, which is GoHighLevel (GHL) set up for Medicare agents.



- [ ] **Follow-up on autopilot:** the first text, the welcome series, reminders, no-show texts.
- [ ] **Every lead in one place:** new, contacted, booked, client, and who you need to call today.
- [ ] **You don't have to remember:** it sends on its own, even when you're at a client's kitchen table.

A CRM with nothing set up inside it is just an empty box. Without it, your ads waste money: leads come in, and nobody follows up.

A CRM without automations is just an empty box.

LIFETIME VALUE

What one client is actually worth.

Say it cost you **$125** in ads to get one client. Here's what that one person can pay you when you do it right. After we explain what Medicare doesn't cover, we offer extra coverage in three sizes: Gold, Silver or Bronze. Below is a Gold package.

MA · GOLD	COMMISSION
Medicare Advantage	$694
Hospital Indemnity pays cash for hospital stays	$400
Critical Illness pays cash for cancer, heart attack, stroke	$600
Home Health Care	$350
One client	**$2,000+**

SUPPLEMENT · GOLD	COMMISSION
Medicare Supplement	$400
Dental	$300
Critical Illness	$600
Home Health Care	$350
One client	**$1,650+**

Same Gold package for a Medigap (supplement) client. Different needs, so different policies. You can sell all of this in year one, or over time.

COST TO GET THE CLIENT

$125

→

WHAT THEY CAN PAY YOU

$2,000+

You're not buying a lead. You're buying a relationship.

SHIELD PACKAGING

The cross-sell is built into the call.

Selling a second policy is called a cross-sell. Most agents try it at the very end of the call. That feels pushy, so it fails. We do it differently. We call it **Shield Packaging.** The "shield" is extra coverage (cancer, heart attack, stroke, home care, dental) that sits on top of their Medicare plan. You set it up from the very first questions of the call, so by the end, it's the obvious answer.

STEP 1

Ask the right questions at the start.

At the start of every call you ask questions about their health and needs (we call this the Client Needs Assessment, or CNA). Ask these, and write down every answer. You'll use them at the end.

> **"Do you have any family experience with cancer, heart attack, stroke, or any critical illness like Alzheimer's or dementia?"**

> **"There are a lot of non-medical expenses that come with a critical illness. Is that a concern for you in the future?"**

> **"What about skilled nursing or nursing home care? Any experience with that in the family?"**

> **"If something major happened, would you rather recover at home or in a facility?"**

> **"Medicare only covers up to 100 days in a skilled facility. If your goal is to recover comfortably at home, we need to address that gap properly. Is that something that would concern you?"**

> **"Are you carrying any dental coverage right now? Is that important to you when getting onto Medicare?"**

You aren't selling anything yet. You're letting them tell you, in their own words, what they're worried about.

STEP 2

Show them the gap.

When you explain Original Medicare (Parts A and B from the government), show them the risk in real dollars:

"While Original Medicare is the foundation, we never recommend staying on it alone. The biggest risk is **20% coinsurance with no cap.** A $100,000 bill means $20,000 out of pocket, with no ceiling."

Then ask: **"Do you have a piece of paper in front of you?"** From here on, they write everything down. People believe what they write themselves.

STEP 3

Price every option with the shield already in it.

This is the move most agents miss. When you give prices, never give the Medicare price alone and add the shield later. Every price you give already includes the shield. **The shield isn't an extra. It's part of the plan.**

OPTION 1 · MEDIGAP (MEDICARE SUPPLEMENT)

Plan G premium	Part D (drugs) about $10/mo	+ Shield $50 / $100 / $150

OPTION 2 · MEDICARE ADVANTAGE

Advantage plan with drugs usually $0	+ Shield $50 / $100 / $150



SHIELD 1

$50



SHIELD 2

$100



SHIELD 3

$150

The shield isn't an extra. It's part of the plan.

STEP 5 · THE HEART OF IT

The 3-Step Roadmap.

Turn one big, scary decision into three small steps. The shield is step one.

> **"Do you still have that piece of paper? Perfect, write this down. Here's how we get everything done in 3 simple steps."**

1

TODAY

Shield coverage

We get your shield coverage set up today for $X.

2

NEXT

Parts A + B

You enroll in Original Medicare (Parts A and B).

3

3 MONTHS BEFORE YOUR START DATE

Your Medicare plan

We enroll you in your Medicare plan for $X.

> **"Today we knock out Step 1, then I'll schedule your follow-up about 3 months before your Medicare start date for Steps 2 and 3."**

WHY IT WORKS

The shield gets sold today, while they're healthy and excited. The Medicare plan comes later, right on time. People turning 65 often can't start Medicare for months. With this, **you get paid now, not in six months.**

STEP 4

Let them learn, then recommend.

> **"Now that we've gone through both options and the pricing, which route do you feel yourself leaning toward?"**

Pause. Let them answer. Then read their own words back:

> **"Based on everything you shared today [family history, recovering at home, dental], my formal recommendation would be [Option 1 or 2] with Shield [1, 2 or 3] for about $X a month."**

THEN **Stop talking and wait. Let them answer first. Then answer their questions and sign them up.**

IF THEY'RE NOT READY

> **"We can still get your cancer, heart attack and stroke coverage approved today, because if something happened next month, I want you protected. Let's take advantage while you're healthy."**

Then email them a short summary: their top two options, the good and bad of each, and the prices. Book the sign-up call for three months before the month they turn 65.

Selling extra coverage is easy to learn. You can sell it all in the first year, or add it over time. I know an agent who ran webinars and didn't make a single sale for six months. He only sold Medicare plans, and those couldn't start until later. Once he added the shield to his webinars, money started coming in right away. People wanted that coverage now. He now brings in $300,000 a month.

You get paid now, not in six months.

YOUR NOTES

Notes: Backend.

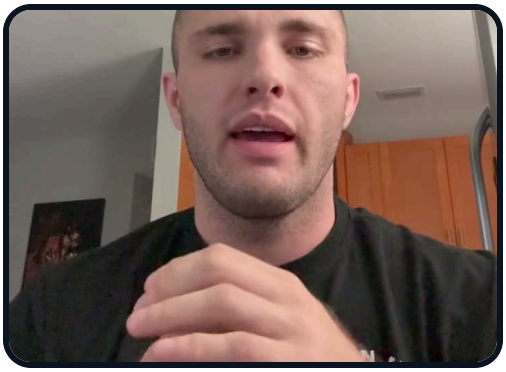
SPEED
How fast am I calling new leads today?

SHOW-UP RATE
What % show up? What will I add?

THE SHIELD
Who in my book should hear about the shield?

AGENTS WHO BUILT THE MACHINE

Real agents. Real results.

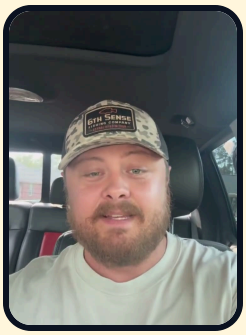


BARRY · AGENCY OWNER

20 → 50

SALES A WEEK

Barry owns an agency with seven people. Since working with Javier and me, he says they went from 20 sales a week to 50, and he doesn't have to run his own ads anymore.



~$12

PER BOOKED APPT

JUSTIN GRAY · ONE NICHE

His ads for deaf clients (people who use sign language) worked so well we turned them down to $1 a day.

Individual results vary. Not a guarantee of future results.

If you keep going, you win. Most people quit, and usually they quit right before it starts working.

Most people quit five feet from gold. Don't.



PART THREE

Local Market Domination

How to become the Medicare person every office in your town knows.

With AI, you can do this faster than ever. This part shows you where the idea came from, how to make a list of local offices in one afternoon, and what to hand them.

06 It started with a phone call

07 Building the list

08 What you put in their hands

CHAPTER 6 · IT STARTED WITH A PHONE CALL

"My dental office told me I needed United Healthcare."

About a year ago, a client called me and said that. I told her that wouldn't work. Her local hospitals didn't take United. So I said: let me call them.

1. **So I called the office.** I asked for the person who handles billing. I explained that the big local hospital system had stopped taking United Healthcare, and what that meant for their patients. I kept it simple: what's good for your office has to be good for your patients too. She was kind. And the dentist was listening the whole time.
2. **She put the dentist on the phone.** We talked for 30 minutes about what his patients were being told, and what it would cost them later. Then he asked me if I'd be a resource for his office. **I never asked him for anything.**
3. **What happened next.** He asked for something to hand patients. That's it. I made a one-page flyer and sent it with my business cards. They've been copying it and handing it out ever since.

Client calls "My dentist said United" → **I call billing** Educate, be nice → **Dentist joins** 30 minutes → **He asks for a flyer** One page + cards → **53 clients** in one year

53

CLIENTS FROM ONE OFFICE, IN ONE YEAR

Some Medicare Advantage. Some regular dental clients.

BE EXTREMELY NICE

"Y'all are telling my clients to switch plans, stay in your lane" is how you lose the office before you ever get started.

I never asked him for anything.

THE WHOLE LESSON

Make it all about them.

Something my brother taught me years ago.

1. **Find a problem the business already has, and solve it.**
   The hospital change was their problem before it was mine.
2. **Never ask for referrals**
   Tell them you're there to help with anything they need or any question they have.
3. **Close by reminding them it changes every year**
   That's your built-in reason to come back.

THE QUESTION

Just like with leads, some offices say yes and some don't. **So how do you do this with lots of offices?** That's the next two chapters.



JOHNNY'S TAKE

"Never ask for referrals. Tell them you're there as a resource for anything they need or have questions on."

Be the resource. Never ask.

CHAPTER 7 · BUILDING THE LIST

This used to take weeks. Now it takes an afternoon.

STEP ONE · ASK AI FOR THE LIST

Open an AI tool like ChatGPT or Claude. Copy this message, put in your town, and send it:

COPY THIS PROMPT

```
"Build me a prospect list for [YOUR TOWN], 25-mile radius. I sell Medicare. I want
dentists, hearing and vision practices, physician offices, pharmacies, home care
agencies, gyms, employers with under 20 employees, employers with 21 or more
employees, insurance agencies that do NOT list Medicare on their website, and
financial planning firms. Include addresses and phone numbers."
```

STEP TWO · MAKE IT DOUBLE CHECK

Ask it to check its own work:

COPY THIS PROMPT

```
"Go back through this list and check whether any of these practices have moved
locations or changed phone numbers."
```

REALITY CHECK

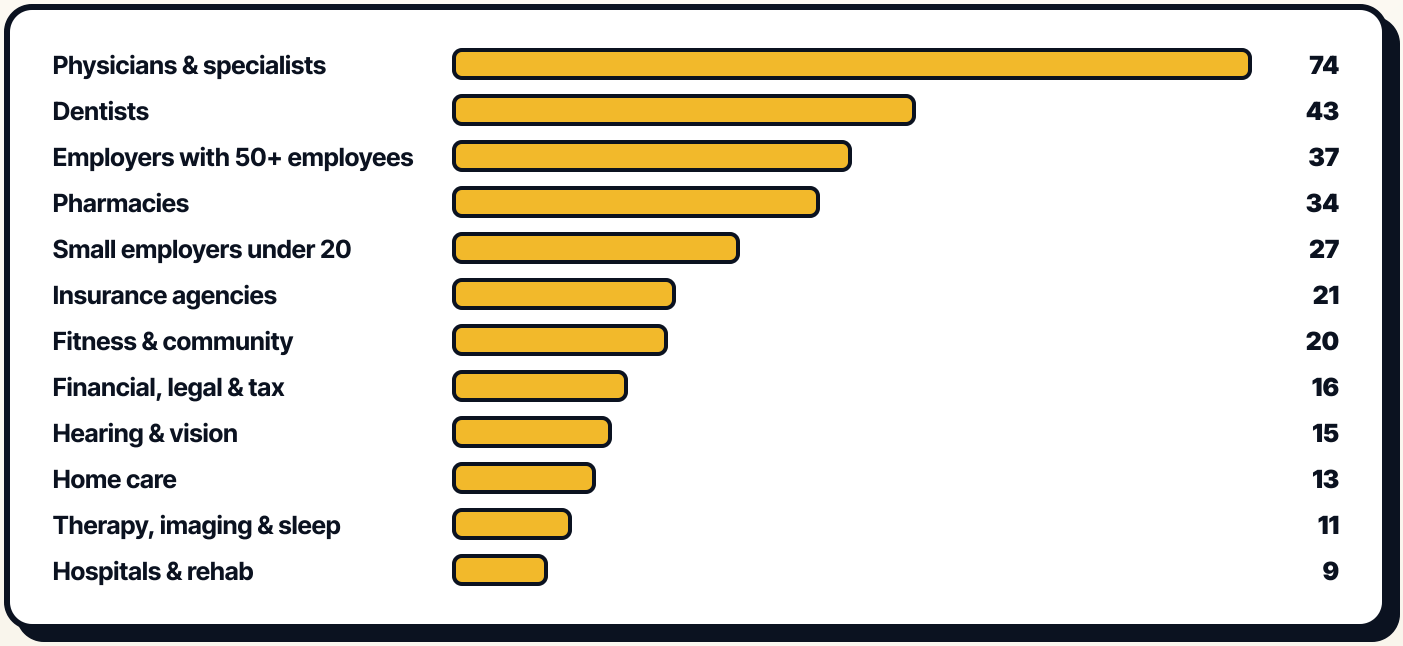
It won't be perfect. But it's a hundred times faster than doing it by hand with a notepad. **Call to double check before you drive anywhere.**

One afternoon. Not one month.

HERE'S MINE

323 businesses in Tupelo, MS.

Tupelo has 38,000 people. In one afternoon I made a list of 323 businesses there. Every one of them sees people who are on Medicare. Here are the biggest groups:



323
BUSINESSES

1
AFTERNOON

Every one of them sees people on Medicare.

THE PART NOBODY EXPECTS

Park once. Work all afternoon.

Sort your list by street address. Medical buildings have lots of offices under one roof, so you can visit many at once.

7 OFFICES IN ONE BUILDING
499 S Gloster St

6 OFFICES IN ONE BUILDING
589 Garfield St

6 OFFICES IN ONE BUILDING
860 S Madison St

6 OFFICES IN ONE BUILDING
1542 Medical Park Cir

FOUR PARKING LOTS

25 offices. One afternoon. No driving all over town.

TRY IT

Ask AI: "Group this list by street address and sort by how many businesses share one."



JOHNNY'S TAKE

"Filter the list by address and the route builds itself."

Park once. Work all afternoon.

CHAPTER 8 · WHAT YOU PUT IN THEIR HANDS

You have the list. Now what do you hand them?

Make a one-page flyer for the office to hand patients. Start with dental, hearing and vision, because every patient asks about those. One page each, with real plans, real hospitals and real prices. You can make your first one this week.

How every one-pager is built.

Every flyer has the same 7 parts, in the same order:

1. **A neutral, professional header**
   A plain title like "North Mississippi: Dental Overview by Carrier, Coverage and Networks." Never say anything bad about an insurance company.
2. **Why the sheet exists**
   A short note: "These are the plans most people here choose. They're not the only options. Talk to your agent about what's best for you."
3. **Which path is the patient on?**
   Original Medicare and Medigap don't cover dental, vision or hearing, so those patients need a separate plan. Medicare Advantage plans include them, but only with certain doctors and hospitals (the "network").
4. **Top Advantage plans in your area**
   A table of the top Advantage plans in your area, showing which local hospitals take each one. Then a smaller table for people who have both Medicare and Medicaid (called "dual" or D-SNP plans).
5. **The sentence that protects everybody**
   A line for the front desk to say: "We can tell you what our office accepts, but we can't tell you which plan to choose. That depends on your doctors and your hospital." Then they hand over the flyer.
6. **"Keep this by the phone" and "This changes every year"**
   Patients with Original Medicare and Medigap need a separate dental plan, so list which ones the office takes. And remind them plans change every January. Tell the office you'll send a new flyer when things change.
7. **Your contact block**
   Your name, license number, phone and email at the bottom.



EXAMPLE · THE TUPELO DENTAL SHEET

The 5 Advantage plans in Lee County, and which local hospitals take them.

This is the main table from the real flyer. "In" means that hospital takes the plan. "Dental max" is the most the plan pays for dental each year.

PLAN	TYPE	NMHS	BAPTIST	MONROE REG.	DENTAL MAX
Aetna Medicare Signature Plus	PPO	In	In	Out, higher cost	$1,500
Aetna Medicare Signature	HMO	In	In	Out, ER only	$2,000
HumanaChoice H7617-085	PPO	In	In	In	$2,500
AARP Medicare Advantage (UHC) MS-0001	HMO-POS	Out	Out	In	$3,000
Devoted Choice 001 MS	PPO	Out	In	Out	$3,500

Notice: the plan with the best dental isn't always the one your hospital takes. Check both. HumanaChoice has middle-of-the-road dental but works at all three hospitals. **Look at the United (UHC) row: that's the exact problem from my phone call story.**

These plans are from Tupelo, Mississippi in 2026. Check the latest plan details every year before you print.

EXAMPLE · THE HEARING SHEET

The key is what the patient actually pays.

PART B COVERS	PART B DOES NOT COVER
Hearing and balance tests When a doctor orders them because something is wrong.	**Yearly checkups or hearing aids** That's where the plan matters.

On every plan we listed, a yearly hearing checkup costs $0. Original Medicare only pays for hearing tests when a doctor orders one, and it never pays for hearing aids. So the flyer shows **"what the patient pays"** for hearing aids on each plan. Dual plans (Medicare + Medicaid) cover a lot more. Some plans give new hearing aids every year, others every two or three years, so tell the office to check.

EXAMPLE · THE VISION SHEET

Vision splits across two systems.

Eye care for a medical problem and a regular eye checkup are covered differently.

MEDICAL PROBLEMS · MEDICARE PART B PAYS

- [ ] Cataract surgery
- [ ] Glaucoma tests for people at high risk
- [ ] A yearly eye check for people with diabetes

REGULAR EYE CARE · PART B DOES NOT PAY

- [ ] Yearly eye exams
- [ ] Frames and lenses
- [ ] Needs a separate vision plan or an Advantage plan

THE ONE MOST PATIENTS GET WRONG: CATARACT SURGERY

Covered: the surgery with a basic lens, plus one free pair of glasses or contacts afterward. Almost nobody knows they get free glasses.

Not covered: an upgraded, fancier lens. That's usually the biggest part of the bill. Tell them before the surgery, not after.

These plans are from Tupelo, Mississippi in 2026. Check the latest plan details every year before you print.

YOUR NOTES

Notes: Your town.

MY LIST
Which 10 offices will I call first?

MY BUILDINGS
Which addresses have the most offices?

MY FLYER
Dental, hearing or vision first?



PART FOUR

The Community

Where to get help, ask questions, and launch with us.

INSIDE THE CLASSROOM

Every part of the machine, taught step by step.

This book is the map. Our online classroom walks you through every step, with videos, and you can ask Johnny, Javier and other agents your questions directly.

01 Introduction	02 Testimonials	03 Reels
04 Recorded Calls	05 Beginners Only	06 Staying Compliant
07 Medicare Basics	08 Extra Policies	09 Sales 101
10 Technology	11 Marketing	12 Virtual Assistants
13 Scripts	14 Annuity Course	15 Facebook Ads Course

WHO IT'S FOR

It's for agents who work with Real Insurance Group. Agents who don't work with us can join too, for **$497 a month.**

QUESTIONS? WE'RE HERE FOR YOU

johnny@therealinsurancegroup.com

javier@therealinsurancegroup.com

You don't have to build it alone.

YOUR NOTES

Notes: Questions to ask us.

STUCK ON
What part of the machine am I stuck on?

ASK JOHNNY
What do I want to ask on the next call?

THIS WEEK
The one thing I'll finish this week:

YOUR MACHINE, ONE PAGE AT A TIME

If you did everything in this book, would you succeed?

I believe you would. Here's the whole book as a checklist. Check one box at a time.

01 · MARKET

- [ ] Pick one type of person and one problem to talk about.
- [ ] Look at the groups nobody's calling: people who worked past 65, people with health problems, people who only want Medigap.
- [ ] Pick one path (like Facebook forms or webinars) and get good at it first.

02 · MESSAGE

- [ ] Every ad has a hook, a problem, a promise and one button.
- [ ] Write in their words, not yours.

04 · FUNNEL

- [ ] Your web page says the same thing as your ad.
- [ ] Short form. After the form, ask them to book a call.
- [ ] Calendar shows only the next few days.
- [ ] Send a text right away from your phone number.

03 · ADS

- [ ] Start with booking ads, then trust ads, then finding ads.
- [ ] One campaign for testing new ads. One for growing winners. Set the money on each campaign.
- [ ] One idea per ad set, one or two ads each.
- [ ] Let a new ad spend what one client is worth before you judge it.
- [ ] Put 70% of your money on winners, 30% on testing.
- [ ] Add about 25% more money a day to winners.
- [ ] Move winners with "Use existing post."
- [ ] Keep making new ads. More money needs more ads.
- [ ] Check the four numbers: spend, CPM, frequency, cost per lead.

Work one box at a time.

YOUR MACHINE, CONTINUED

Back end and your town.

05 · BACKEND

- [ ] Text in 1 minute, call in 5.
- [ ] Welcome texts and emails for the first few days. Then one helpful message a week for months.
- [ ] Reminders the night before and the morning of. If they miss it, text and rebook.
- [ ] Every week: fix your worst number. Every month: try one new thing.
- [ ] Offer the shield to every client.

LOCAL MARKET DOMINATION

- [ ] Use AI to make a list of offices within 25 miles, then have it double check.
- [ ] Sort the list by address and plan which buildings to visit.
- [ ] Make your first flyer (dental, hearing or vision) this week.
- [ ] Call the office, ask for billing, and make it all about them.
- [ ] Be the helper, not the salesperson. Come back every year. Send flowers.

Market + **Message** + **Ads** + **Funnel** + **Backend** = **Clients**

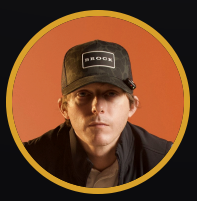


FROM JOHNNY AND JAVIER

If you did everything in this book, you'd be successful. Build it one part at a time, and we'll see you inside.

Every part does its job, every month.

REAL INSURANCE GROUP



Johnny Brock



Javier Martinez

Your ad doesn't make you money. Your machine does.

Who you talk to, what you say, your ads, your pages and your follow-up. The exact system Johnny Brock and Javier Martinez use to turn Facebook ads into booked Medicare calls, and calls into clients worth $2,000+.

REAL INSURANCE GROUP · A BROCK PARTNER